MENTOR REVIEW · WORKING PROTOTYPE

The right hospital guidance. At the moment it matters.

A caselog companion that connects encounter context, hospital rules and source evidence.



Interactive caselog | Cited guidance | Local deterministic core

Illustrative Synapse workflow companion. Synthetic demo; no clinical or production access.

Hospital rules change.
The encounter keeps moving.

How can a provider find and apply the correct local workflow without repeatedly leaving the caselog?

CONTEXT

Different hospitals

Ordering responsibility, video location, drug preparation and completion rules vary.

TIME

Different coverage

Staffed windows, overnight schedules and hospital timezones change the relevant action.

TRUST

Different source quality

Missing, stale or conflicting instructions should remain visible rather than turn into confident answers.

Design problem derived from the supplied challenge and synthetic hospital guides; no patient-outcome claim.

Record once.
Review guidance in context.

01

Caselog

Editable encounter fields with guidance beside the relevant step.

02

Hospital knowledge

Original-source citations, grounded questions and visible quality flags.

03

Review and evaluate

Explicit completion gates, unfamiliar hospital imports and independent scenario checks.

INPUT

Encounter context

→

GROUNDING

Pinned hospital guide

→

OUTPUT

Action + evidence + uncertainty

One encounter. Six working stages.

01 Intake → 02 History → 03 Imaging → 04 Recommendation → 05 Note → 06 Sign-off

Start with context

Hospital, consult type, encounter type, guide version and explicit local time.

Guidance follows events

Opening steps, editing fields, focusing order context and requesting completion.

Rounding has its own branch

Days, timing and order responsibility come from rounding sources.

Workflow navigation is flexible. This prototype does not enforce every caselog field before moving on.

Every useful prompt has a source.

EVENT
Field edited

→

CONTEXT
Hospital + step + time

→

RULE
Deterministic evaluation

→

EVIDENCE
Cited guidance card

Proactive when relevant

LVO, CTA, unknown last-known-well, allergy, orders, EEG, notes and rounding.

Quiet when irrelevant

General Neuro with allergies “none” produces no guidance in the deliberate silence scenario.

The clock is part of the workflow.



Hospital-local decisions

Explicit instant + IANA zone. Overnight and weekday coverage are evaluated together.

SOURCE HEALTH

Uncertainty is visible

Stale, conflict, missing verification and interpretation-review flags preserve what we do not know.

Stale · Conflict · Schedule unknown

QUESTIONS

Grounded answers

Resolve the field and requested time. Search this hospital/version. Cite a source, ask clarification or show absence.

Confirmation must be explicit.

REQUIREMENT UNMET

Block and explain

Identify the charge or provider-confirmation requirement, cite its source and link to the field to fix.

REQUIREMENT RECORDED

Reevaluate and complete

Evaluate the latest revision. Record mock completion only after supported gates are satisfied.

REQUEST

Complete

→

CHECK

Source + explicit field

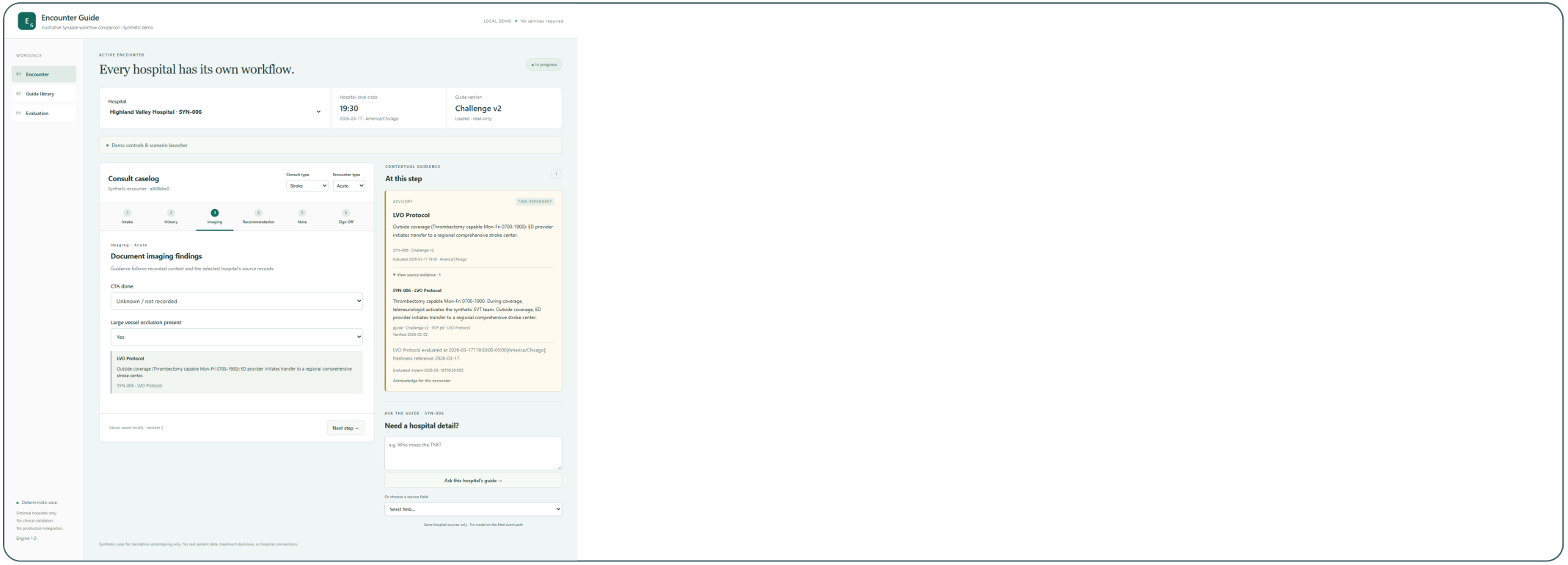
→

RESULT

Fix blocker / mock success

Unverifiable requirements show uncertainty and do not create invented hard stops. No EHR note signature or charge posting occurs.

Let's use the prototype.



Live app is fully interactive. Screenshot is a recorded local verification fallback, not a recording of the current hosted session.

A new hospital.
No rebuild. No restart.

01 JSON / CSV → **02** Validate + preview → **03** Activate version → **04** Select / Refresh → **05** Cited guidance

Unfamiliar fields work

New interpreter instructions become searchable; declarative mappings add a field and proactive card.

Invalid imports stay inactive

Atomic validation retains known-good data. Existing encounters stay pinned until Refresh.

Working behavior. Inspectable results.

29 / 29

Sponsor examples

20 grounded questions + 9 proactive scenarios, including deliberate silence.

74 / 74

Unit checks

Sponsor acceptance, unfamiliar hospitals, time/data-quality edges and triggers.

10 / 10

Browser workflows

Production build, imports, gates, mobile/keyboard, offline loaded core and latency.

30.1 ms field-change p95 against the prototype's 500 ms target.

Recorded local Chrome production baseline; 120 field-change samples. Not a hosted-network benchmark or clinical validation. Full test evidence

Shared code. Reproducible review.

CONTRIBUTE
Fork / branch

→

VERIFY
Tests + build

→

REVIEW
Pull request

→

SHARE
Hosted demo

No paid API or hospital credentials needed for the deterministic core.

Case drafts and imported guides are browser-local; the demo has no shared clinical database or collaboration backend.

What should we validate next?

WORKFLOW

Relevance and restraint

Which prompts help at the point of work? Which should stay silent or require explicit review?

GOVERNANCE

Guide ownership

Who verifies local rules, resolves conflicting sources and approves changes?

INTEGRATION

Production boundary

Authentication, audited storage, host events and clinically validated failure policies need separate design.

Today: a runnable, cited, testable synthetic workflow prototype.

Finite rule/question grammar. No LLM, speech, real EHR write, patient use or production safety guarantee.